

I: Okay, so then let's start with the actual questions. The first question is about familiarity with AI. How familiar are you with AI technologies in general? There are two questions with a score, and this is one of them, but I also just want you to talk a bit openly about how familiar you are.

P: I'm familiar with AI but in Portugal we don't use it much. We know the concept, but we don't use it in a practical way in the interview or in the evaluation. We use it more like a tool in a very psychoeducational way. If we have to score it, I think it's an eight on a scale of zero to ten. Because I'm familiar with it, but not completely, I think because of my country and the way that people see psychological treatment. It's very one-to-one, you know, it's very important to establish a very early alliance. It's a very therapeutic relationship. So many people, for example, are not comfortable with online interviewing or counseling. That only began in Portugal because of COVID. Sorry for my English.

I: So it's very focused on human-to-human connection and the online interactions have only started with COVID. So because people can see each other, they started using online e-health, right?

P: Yeah. And for example, I work in a hospital focused on cancer, and my work is in palliative care. So it's very difficult to use AI. We only use AI in a virtual way. For example, I have a patient who has a dream to travel to Australia. And we can put on, I don't know how to say, like the glasses, you know?

I: Like the VR headset?

P: Yeah. And we manage just to check the list of things they want to accomplish before they die. It's like a bucket list. That's the only way I use AI in my work.

I: Okay. So has your organization introduced any AI tools or systems?

P: Yeah, I think we can say yes. For example, we have a platform, mostly for breast cancer. And people fill in a form, it's like a questionnaire about how they feel post-surgery, about self-image or relationships with their partner. And based on the responses our patients give, the platform provides them with flyers or contacts, for example, for my unit, the psychological unit, and they can call us. But it's limited to that, not much more. So only for breast cancer.

I: So they get some kind of questionnaire, and how is AI used in that?

P: For example, it scores a average of everything, and after that I think there is a question where it asks, do you want to chat a little bit, or do you need some advice, or do you need something? And then things pop up based on that.

I: So there is a questionnaire, and based on how you respond, there is some AI that's analyzing your responses, and then based on your responses it might say, oh, here's a flyer about whatever, or here is some contact information for psychologists that we are connected with.

P: And this flyer is a flyer that we make and put there. Or, for example, some training programs, I don't know how to say because it's a Portuguese thing. It's like Liga Portuguesa Contra o Cancro. It's like an organization that offers some training or educational support for people with cancer.

I: But when you use AI, your clients are also aware that you are using AI, or did you not tell them?

P: No, we tell them because it's like a consent. When the flyers appear we tell them. So when the form and everything appears in the beginning, we tell them that their responses may generally suggest some things that AI is going to provide.

I: So there will be like a pop-up that says your responses will be checked by AI?

P: Yes, because it's an ethical requirement we have in our hospital. The people, before they respond to the form, have a consent that the suggestions are generated based on their responses, under our supervision.

I: What are your thoughts on AI being used in diagnostic or assessment processes?

P: That is a tricky question, because as I was saying, AI for me is like a tool. Not something that you can let manage things. For example, if the results say she has depression or anxiety, it's like a tool. For my interpretation, for my judgment, for my clinical knowledge, I still decide. But it's a tool for me.

I: So you think AI is more of a tool rather than something you can base your judgment on, right. You still have to make your own judgment as a clinician and not just say, oh, what does the AI say, it says this person has anxiety disorder, we accept, right?

P: Yeah. For example, in my private work, I have some patients who searched in Google or ChatGPT, what is vulnerability? Because I told them vulnerability is something we can work on. And sometimes it's very difficult for me as a psychologist, because it's like a risk, and sometimes it's educational, it depends on the way that the patient searches for things. But yeah, I think it's a tool, but not something where you can say, she scored a seven, so she has an anxiety problem or depression or comorbid depression or whatever.

I: So you mentioned an example where a client of yours searched ChatGPT to check what vulnerability was, because that was something you mentioned in treatment that they should work on. And there is some risk there that sometimes the information ChatGPT gives might not be correct. But there is also an opportunity, because sometimes it might be correct or it might be helpful for that person to understand, oh, this is what vulnerability is. But it could not be used in a way where if someone has a result of seven on some scale, this means they have this disorder, right. Do you currently do any intake processes? So for example, the first session where you talk with your client and ask about their problems, this is called the intake. Do you do an intake with your clients, and do you think AI could help in this intake?

P: I think no, not for the first sessions. Because, for example, I have a patient who is not Portuguese and is very open-minded about AI or something more creative, but in my opinion the first sessions are very important to establish a therapeutic alliance. So if you introduce a tool in the first session, I think it disrupts that. The relationship is not built in a fluid way. But I think from the third or fourth session onwards, for example when you start with exercises or something that can help patients monitor things, for example anxiety triggers, that's when I use monitoring exercises to help people notice the triggers, how it starts, what thoughts it brings, when it starts, what they think, how they feel, what the situation is. It's more of a cognitive structure. But in the first sessions, no, I don't use it.

I: So you think the first session is really important to start working on that therapeutic alliance, right?

P: Yes. Because when you choose to start a therapeutic process, you are very nervous. Sometimes you don't know where to start, what you're going to say, are they going to judge me? My patients talk to me after the first session and ask, now that you know me, what do you think? What do you feel? So if I give them a tool and not a person, I think they're going to be defensive. So for me it's not something I use at the start.

I: So you think the first sessions are very important to build that alliance, and if you introduce an AI system there it will block some of that openness and people might become defensive. You also mentioned that clients ask for your personal judgment on their situation. But you say you could maybe use it later, like from the fourth or fifth session, when you're already starting to do exercises. For example if you're doing CBT or whatever, there are specific treatments where maybe you can use it in explaining exercises or doing exercises with some AI support.

P: Yeah, because in the first sessions you get to know the person. You understand what's important for that person, what matters to them. And after that, I would say you have a tool where, okay, I'm a psychologist, I know the person, but sometimes the tools I have cannot match what they need. And AI is a tool that can sometimes give you a connection between what you know about the person and the tool you want to give them, but sometimes you have to make it personalized.

I: So also with those first sessions, you figure out how this person might react to AI and you maybe need to adjust it to make it fit better, or maybe it doesn't fit at all. And then you can decide, okay, I don't think this tool will be useful for you, right. So have you considered AI for preliminary screening? This is like the screening before the intake, right. And you kind of mentioned that you already do some questionnaires where AI is being used. So can I assume that you are already using AI for preliminary screening?

P: Yeah, in my public work, yeah. Because it's something we have in the hospital. Me and my colleagues use it from the first contact, because it's not only psychologists who have access, doctors and nurses do too. And we combine the symptoms and the responses they give and we talk to each other, okay, this person maybe it's better to have an appointment with a

psychologist next week, or the doctors have to discuss, for example, a procedure, things that we manage as a team.

I: How do you view the role of AI in tracking patient progress over time?

P: I think I touched on that a little bit, but I think it's very relevant, for example, when I use it more with anxiety, or to help people understand how they feel in similar situations. The one I use most right now is for sleep. There are exercises, or for example the watch that people use during sleep, and you monitor the heart rate and the hours of sleep, and how they feel throughout the day. That's what I'm working on most right now. Sleep, cardio, and anxiety, because they are very interconnected.

I: So that's kind of the scope in which you think AI could help in monitoring, right. What concerns, if any, would you have about AI tracking patient progress?

P: It's like the thing I mentioned recently. For example, I had a meeting just before this one with a patient and she told me that over the weekend she did sports, she ran, she had time with her family, things she wanted to do, and that was okay. But her sleep score or her anxiety score was very high. She felt very uncomfortable over the weekend. And while we were talking about the monitoring, what do you feel, what do you think, she told me she went to ChatGPT and asked, is something wrong with me? Am I doing everything right? And this is the risk, because we are in a process. I told her, if you need anything, please send me a message or call me and we can talk a little. But she preferred to write the question to ChatGPT. And today she told me, maybe it would be useful to talk to you, because what I see there is not what we are doing. And this session is about self-love. We have to recognize the concepts and accept that sometimes we don't have a lot of progress or the response we want, but we have to keep moving and find our way there. But sometimes you have one step forward, two steps back. So this is the risk, because for example if she calls me and I don't answer, she can always write a question and ChatGPT will respond. But sometimes it's not personalized. ChatGPT doesn't know the person who is asking. So sometimes the person can think, okay, this is rational, this makes sense, but for me it does not make sense. And sometimes you cannot work through that. So it's very difficult in the process.

I: So one of the possible concerns is that with progress tracking, or in general with monitoring, if your client becomes anxious or wants to know something about how it's going, maybe their sleep score is bad that day or their anxiety score is bad, and they immediately go to ChatGPT asking, is there something wrong, what is going on, then there's a possibility that ChatGPT will not know the full context of this person and also not the full context of the treatment plan. And then it might give wrong answers or answers that are not helpful in reducing that anxiety. Where instead of going to ChatGPT, you would prefer them to come to you because you know their context. And you could just tell them, okay, sometimes you will just have days where your sleep or your anxiety is worse and that's fine. We will slowly move forward in this process, we might go two steps forward and one back, but you just have to trust the process. And there is a chance that patients will use AI to increase their anxiety because they want to check how it's going, right.

I: How do you feel about AI generating drafts or reports or assisting with scheduling and reminders?

P: I don't use AI for that, it's me doing it. But I can see a potential, for example, AI sending the email or the message for a new meeting or session, so we have more time to prepare the session or prepare some exercises. But I prefer to do it on my own.

I: So you prefer sending that yourself, or you would be open to AI doing that for you?

P: I'm open to it, but right now I do it myself.

I: So how much time do you think this could save you? Can you give an estimate?

P: I don't know, but for example tomorrow I have 15 patients. If I don't send them reminders, maybe around one hour a day.

I: What are your thoughts on using AI for psychoeducation or therapeutic exercises between sessions?

P: I think it's okay. I don't use it myself, but colleagues from my hospital use it to create content for Instagram or social networks. For example, they have an idea for a post about cancer and the psychological impact, but they don't know how to structure it, so they ask AI to help. I don't do that, but I understand that sometimes for the creativity or the structure it helps. I use it more for educational purposes, for exercises or monitoring, but not a lot. I think it's maybe not something I should say, but for example the patients I have who are engineers, sometimes they are more engaged if I tell them, okay, let's do this, I do this with AI. And they get very excited and more engaged in the process.

I: So you mentioned that some of your colleagues have used it for social media content creation, when they want to make a post about cancer and the psychological effects. And you also mentioned that you use it sometimes for exercises and monitoring, and you always mention it when you use it. And you explicitly use it more with engineering-focused patients because they are more excited about AI, so to make them more engaged in treatment you introduce AI. Could you explain an example of an exercise you do with AI?

P: For example, there's an app, I think some colleagues introduced it to me, it's called Wysa, W-Y-S-A. So for example, you have a thought, you have a feeling, and they put it into motion. For people who are very introverted, it's very difficult to know how they feel with anxiety, because the thoughts don't stop, they are constantly racing. So you put the thought in and it has an emotion attached to it, and there are exercises that can help you reduce the anxiety. For example, to bring you to the present moment, we do a grounding exercise. With your breathing, we ask patients to think of five things they see right now, four things they hear right now, three things they can touch. And then they do the exercise without me having to be there. Afterwards they tell me, okay, I did this exercise. How did you feel, is the anxiety better or not, do you feel anything else? And it's like a supervision for the practice.

I: So where does AI come into this? The exercises already exist, right?

P: Yeah, but for example a person who has a symptom and they express to the app, okay, I'm feeling this, my anxiety is overwhelming, please help me, and then it provides a breathing or grounding exercise.

I: So let me check if I understand. There is this type of exercise that you do with anxiety patients, where their thoughts are racing and they're not even sure how they are feeling. And there is a grounding exercise where they get through AI, they just say, oh I'm feeling this or I'm very anxious, and then the AI provides a grounding exercise, for example think of five things around you, four things you hear, and so on. And because their attention gets directed onto external objects, it kind of goes away from their anxiety. So afterwards they check how their anxiety is and most likely it will be lower. And the AI is just giving exercises to the patient who is asking for it, right.

P: Yeah.

I: What risks or concerns do you have about using AI in clinical work? You mentioned already a few, like losing the therapeutic alliance and trust, and also that AI might give wrong or unhelpful information because it doesn't have the context of the specific client. Is there any other limitation or risk you want to mention?

P: I think the only thing is, it's like a tool. I don't know how to explain. Because sometimes it's very difficult to talk about it, I don't want to be rude, but it's like it's not going to replace a human, you know? And sometimes I think people think AI can replace a human, and it can't. In a psychologist's work I cannot imagine this is happening, so that's a risk that I'm very concerned about sometimes.

I: So what you mentioned is that AI is not going to replace psychologists, right. It can be used as a tool, but it will never be the case where a patient is talking directly to AI with no one on the other side.

P: Because it's not about interpretation. It's human. You need feelings to talk about feelings. It's a human connection. Like what I was saying, the first sessions are very important to establish a relationship. And if you are talking with a machine, sometimes it's very difficult to try that. And sometimes I'm afraid the machine gets too real for some patients. It's not human, but for patients it gets very real to talk with ChatGPT. That's sometimes my concern, that people don't understand that it's a machine and cannot replace humans.

I: Do you think there's a possibility of some clients becoming over reliant on AI? Like they just get this idea that AI is some person, or maybe not even that, but they just become so engaged.

P: Yeah. People can get attached. For example, they told me in the session, okay, I have a doubt, I asked ChatGPT. I don't talk to my mom, I don't talk to my father, I talk to ChatGPT. And it's very horrifying to be a psychologist and hear that. I'm very scared about it, so I think this is why it's very difficult for me to see AI with good eyes.

I: How do you think clients respond to AI being used in their treatment? You don't think it's that good to introduce it, right, and you also think there is a chance for them to become over reliant on it. Could you also give a score? One being they will not like using AI and ten being they would like it.

P: For example, my clients that are middle age, like 30 or 35, I think they are okay with it. Like a six or seven. But if they are older, no, they refuse. For example, even exercises, I have to do them in sessions. For older people I have to do it in session, but for middle age I can give it as homework and after we discuss it in a session.

I: So you think younger people are more open to it, maybe a six or a seven, but older people might not be that open to it.

P: Yeah, in Portugal. Because for example, the older clients I have who are not Portuguese are very open-minded about AI or they talk to me about AI and other things. For example, I have a patient who loves to travel alone and he told me in the session that he planned three days somewhere with the help of ChatGPT. So could you give maybe an average score for the whole population?

P: I think six.

I: Which aspects of your work should remain strictly human? And what would you need to feel confident about using AI safely in practice?

P: I think things like our evaluations, our assessments, for example. In Portugal we don't say diagnosis. We do an assessment, a document, and you give it to a doctor or to the workplace so the person can have some benefits from it. And I think, it's a tricky question for me because I have concerns about AI that I told you about. But maybe if more people are using it and there are studies that confirm it works, then maybe I would consider using it more. But I don't know. It's a very tricky question, because I think what I do, only a human can do. Like talking to a person, feeling what the person feels, being impacted, being there for the person. It's not just a yes, yes, doing. So it's very difficult for me to imagine. But I know, for example, the things I talked about, like the scheduling or organizing my agenda, those could be a tool. But I don't know how to go beyond that.

I: So what should remain strictly human? You said the relationship, and also the assessment, because you think that's really important and it's all based on the therapeutic alliance, right. And then for you to feel confident using AI, it's quite hard for you to imagine, because your general view of using AI is not that positive. You don't think it could be used in a way where it replaces the human, right. But if you should mention something, maybe it's just that more people are using it, there's more scientific proof that it works, and then maybe you would consider it more. But still you think that in practice the real world scenario will just be that you have to talk with the client.

P: It's more like an ethical thing to think about. The concept of ethics is very important in my profession. So it's very difficult to see AI as ethical. It's about privacy. The risk to privacy, the notes, it's very concerning to me.

I: Do you want to add anything?

P: No, just to say sorry for my English.